

What's in Scope for Interactive Health?

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Over the last decade, Health has emerged as a vibrant area of focus within HCI. Since 2024, the Health Subcommittee has grown by nearly 30% annually, making it one of the largest subcommittees with over 500 papers being processed for CHI 2026. This growth has led to multiple attempts to found a Health and HCI conference, and finally in July 2026 we will celebrate the efforts of the community in the inaugural SIGCHI sponsored **Interactive Health Conference** in Porto, Portugal.



THE FIRST INTERACTIVE HEALTH CONFERENCE IS COMING JULY 2026!

The launch of a new conference naturally prompts excitement and speculation about how it might evolve and create a unique identity of its own. A central issue is what will be considered in scope for the venue and how this will build on the gaps in or be differentiated from existing venues, in this case we consider the CHI Health Subcommittee. As Subcommittee Chairs (SCs) for the **CHI 2025 Health Subcommittee**, and subsequently members of the Interactive Health working group, we were well placed to reflect on community expectations of what falls within or outside the remit of Interactive Health and how it would be differentiated from CHI, or the motivations of doing so in this new venue. More importantly, we consider how to welcome and support the community to foster scholarship at the intersection of HCI and Health.

To contextualise this discussion, the Health SC received a large volume of submissions in 2025, but also recorded a desk rejection rate of 19%. For CHI 2026, with the addition of the Assisted Desk Reject criteria, we saw that number grow to about 31%, a figure we view with ambivalence. These experiences motivate a clearer articulation of scope for Interactive Health and a complementary commitment to inclusive practices that enable high-quality, timely, and field-shaping contributions.

The Subcommittee Process and Decision Making

As background, we first explain how the desk rejection process took place in 2025. Each of the three SCs reviewed a third of the submissions. Submissions considered for Desk Reject or Transfer were flagged and reviewed by all non-conflicted SCs and at least one Paper Chair. Transfers also involved communication with the destination subcommittee. For the 19% that were rejected, reasons ranged from template or anonymization issues to a lack of context in HCI literature. About 10 papers were ‘transfers’ – moved out of Health and a similar number brought in, also involving discussion with another subcommittee. Transfers often reflected misalignment between the intended contributions and the subcommittee’s expertise. Most of the remaining Desk Rejects had issues of “Context” – a new CHI 2025 criterion requiring submissions to engage with HCI scholarship, not just health or computing more broadly. In 2026, CHI’s Desk Reject process was further expanded with an “Assisted Desk Reject” stage, introducing criteria such as insufficient literature reviews, methodological details, or data. The Associate Chairs (ACs) also played a role in identifying issues, from anonymity violations to contextualization issues in 2026. These typically resulted in Desk or Assisted Desk Rejections after being confirmed by SCs and Paper Chairs.

So, What Falls Out of Scope?

Reflecting on the process, we identified three main reasons that work can fall out of scope for Interactive Health, ordered roughly by primacy.

Not Speaking to HCI Scholarship

Given the now-established history of HCI scholarship in Health, we expect that any new contribution to the space would need to engage with prior community conversations around the topic. While we held the bar intentionally low (we considered citations from any SIGCHI-sponsored conference counted), many submissions lacked even minimal engagement with the few that they did cite. We made exceptions for novel Health topics where HCI precedent might be sparse, such as health conditions with emerging clinical definitions or studies of novel technology that is not widely available. But often even these missed existing related literature to help ground the health innovation in user or human-centred concerns.

Prioritizing Clinical over HCI Contributions

We often saw a mismatch between the framed contribution and the focus of the work. Some submissions focused heavily on clinical outcomes or usability in medical contexts, with minimal attention to HCI concepts like design or user experience. The line between HCI and clinical contributions was a recurring point of debate. Even among SCs and ACs, interpretations of what constitutes a sufficient HCI contribution varied. Submissions that primarily advanced clinical knowledge without design or interaction implications were often Desk Rejected or transferred to another subcommittee.

Non-compliance with Community Norms around Anonymity and Formatting

As a community, CHI holds strict requirements about the anonymity and presentation of submissions, such as requiring that all submissions be anonymous. This led to authors having difficulty citing work in third-person (e.g., “as described by Foong et al.”, versus “as described in our prior work”), resulting in them being unable to distance their contribution from their larger Health and HCI projects that are part of clinical trials or a multistakeholder investigation. In aiming to abide by these policies, we found that some authors fully anonymized references critical to differentiating the contribution, which made the work difficult to review. On the flip side, other authors left mention of specific university review boards, grant numbers, or included links to larger project websites which clearly identified the authors or their institutions. Finally, we Desk Rejected a few submissions which were egregiously out of format, largely stemming from challenges following the LaTeX-centered workflow of ACM publications.

As SCs for CHI, our key insight is these scope disparities arise and are exacerbated when we invite interdisciplinary authors to engage with our existing norms. These authors who are trained on the formatting, contribution and review expectations of other venues that publish health, computing and user-evaluation papers (e.g., JMIR) seemed to be the most affected. In other words, the lack of engagement in HCI papers and the focus on clinical contributions suggested that they were too unfamiliar with what the HCI sees as a valuable contribution such as insights about usage, users, and learning about systems.

Looking ahead at the Future of Health and HCI

As we now invite interdisciplinary authors through the explicit linking of interaction with Health, we should also specify what we DO want to see grow at the intersection of technology with the field of health, wellness and care.

Building on these observations, we offer three suggestions, or perhaps provocations, to where the field’s scholarship and community should go. These changes are undoubtedly a shift from what HCI scholarship and community currently prioritizes. But we view these as critical to achieving the

goals of having our scholarship be in conversation with other work at the intersection of technology and health.

1. Profile the Relevance of Health Contributions to HCI

Our experiences highlight a key question about where the field's scholarship should go: Should venues for Health and HCI like Interactive Health enforce traditional HCI norms, or expand to accommodate interdisciplinary submissions from fields such as Medicine and Public Health? If future Health and HCI venues lean too far into being an HCI-only venue, it risks alienating valuable contributors from our adjacent disciplines. Yet too much deviation might weaken the venue's identity within SIGCHI. Striking a balance between inclusivity and disciplinary grounding will be essential.

To this end, we believe that Health and HCI venues like Interactive Health need to relax certain HCI norms, such as requiring deep HCI engagement, towards encouraging thoughtful interdisciplinary dialogue. In return, authors may be asked to profile how a health-focused research could be relevant to the field of HCI. HCI tends to prioritize contributions to HCI theory, methods, or design and innovative applications. But in Health contexts, important work may focus on outcomes, implementation, economics or policy, areas often undervalued in HCI review.

By doing so, the HCI and Health community may need to broaden the definition of contribution to embrace the wider field of health concerns. Recognizing how HCI is implicated and affected by clinical outcomes, system-level insights, or public health impact alongside traditional HCI outputs would make the venue more relevant and inclusive. Additionally, work is needed to draw the explicit connections between terminology in Health and HCI that share largely similar intent, but have different labels (e.g. program design versus design research). This may require new guidelines for reviewers and authors, as well as a broader sense of who belongs in the community. For example, researchers from informatics or digital medicine may engage deeply with technology but not in HCI terms.

2. Relax Publication Norms that Hinder Interdisciplinary Work

Our community's typical anonymization and formatting rules often confuse or disadvantage Health researchers used to other academic traditions. Violations are typically unintentional, stemming from different expectations; or technical, stemming from challenges adopting tools for writing in computing (e.g., LaTeX).

To this end, we believe that Health and HCI venues like Interactive Health need to rethink our submission norms, such as requiring complete anonymity and having strict formatting, towards encouraging thoughtful contributions from a wide range of scholars.

Moreover, we might consider structural changes, such as introducing structured abstracts (common in Medicine) or returning to word limits rather than rigid templates. Such moves could make the work more approachable to health researchers and ultimately improve its impact. To foster broader participation, conferences like Interactive Health should explore alternatives to the standard review model. Health conferences often favor inclusivity (e.g., panels, short abstracts) over rigorous peer review. Introducing flexible tracks could help bridge disciplines, promote conversation over competition, and invite new collaborators to this field. Besides, the shorter, diverse formats could accommodate the rapid pace of emerging technologies to complement rigorous, longer-form work by enabling timely dissemination and debate at the field's frontier, including attention to ethics, appropriate methodologies, and new modes of collaboration. Beyond reviewing, adapting how we define "participation" could increase impact. Not every contributor needs a full paper — sessions for practitioners, panels, or workshops could welcome diverse voices without the friction of formal publication, including patients and non-traditional HCI conference participants.

3. Maximizing HCI in Health via Interdisciplinary Dialogue

Another trend that we observed is the explosive growth of AI-linked papers. This growth has come on the tail of the rise of consumer health technologies, such as wearable devices and mobile apps. We have also seen deep integration of technology into clinical practices for notetaking, care coordination, and diagnostics. Technology is further utilized in public and global health contexts for promoting understanding population impacts and mitigating the impact of social determinants of health.

However, we are also seeing traditionally biomedical health venues such as Nature or the Journal of the American Medical Association (JAMA) cover these topics. For example, an editorial in JAMA describes this new interest in "*how new AI technologies can improve medicine and health for patients, the public, clinicians, health systems, and more.*" The editorial goes on to list the thematic priority areas in AI as clinical care and outcomes, patient-centred care, health care equality, fairness in AI algorithms, medical education and clinician experience, and global solutions that address low resource settings.

As a field that has long engaged deeply in the design and development of technological innovation, this call from the Medical community suggests that we in HCI have not been successful at fostering the sorts of translation of findings and methods that we aspire to have. These areas outlined are core to our research and identity, but viewed by those in medicine as under-explored. We therefore need to do more to make others aware of our community, and establish ourselves as recognized experts on these topics.

So one final direction we hope to see, when we bring our colleagues from Health to Interactive Health, is to see interdisciplinary dialogue, towards the recognition of the value of HCI researchers in the design of Interactive Health, and to see HCI research find impact in life-saving, wellness enhancing implementations of digital health systems.

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